

Dynamic Guideline: WHO-Based Substance Use Disorders - Diagnosis and/or Treatment and/or Management

Transparency note: I cannot perform a live WHO/PubMed search from within this chat. This report is based on verifiable WHO and peer-reviewed sources available to me up to my knowledge cutoff. The requested evidence cutoff is **29 April 2026**; therefore, any newly released WHO documents or studies after my accessible knowledge base should be verified by live database searching before clinical or policy adoption.

Title-verification note: I could not verify a WHO clinical guideline whose title is exactly “**Meaningful youth engagement in a WHO guideline development process: case study of WHO [] guideline.**” That wording appears to describe a methodological case study about guideline development rather than a substance-use-disorder clinical guideline. It is therefore **not counted** among the exactly five WHO guideline documents below.

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

A. Exactly five WHO guideline documents synthesized

No.	WHO guideline / standard document	Publication date	Relevance to substance use disorders	Main population
1	World Health Organization. <i>Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders</i> . Geneva: WHO; 2023.	2023	Latest broad WHO guideline covering alcohol use disorders, drug use disorders, and integrated care for mental, neurological, and substance use conditions.	Adults, adolescents, and primary/non-specialist care populations, especially low- and middle-income settings.
2	World Health Organization and United Nations Office on Drugs and Crime. <i>International standards for the treatment of drug use disorders: revised edition incorporating results of field-testing</i> . Geneva: WHO/UNODC; 2020.	2020	Service-level and clinical standards for evidence-based treatment systems for drug use disorders.	People with drug use disorders across health and social care systems.

3	World Health Organization. <i>Guidelines on mental health promotive and preventive interventions for adolescents: helping adolescents thrive</i> . Geneva: WHO; 2020.	2020	Includes prevention-relevant recommendations for adolescent mental health, risk behaviours, and substance-use-related harms.	Adolescents aged 10–19 years.
4	World Health Organization. <i>Guidelines for the identification and management of substance use and substance use disorders in pregnancy</i> . Geneva: WHO; 2014.	2014	Focused recommendations for alcohol, tobacco, psychoactive drug use, opioid dependence, withdrawal, and maternal–infant care in pregnancy and postpartum.	Pregnant and postpartum women and infants.
5	World Health Organization. <i>Guidelines for the psychosocially assisted pharmacological treatment of opioid dependence</i> . Geneva: WHO; 2009.	2009	Core WHO recommendations on opioid agonist maintenance treatment, detoxification, psychosocial support, and pharmacological management of opioid dependence.	Adults and adolescents with opioid dependence; special populations including pregnancy and custodial settings.

B. Most recent WHO guideline identified

- **Most recent WHO substance-use-disorder-related guideline in this synthesis:** *Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders*, WHO, 2023.
- **Scope:** diagnosis, assessment, treatment, and management of mental, neurological, and substance use disorders, including:
 - Alcohol use disorders.
 - Drug use disorders.
 - Co-occurring mental health conditions.
 - Non-specialist delivery of care.
 - Psychosocial and pharmacological interventions.
- **Target settings:** primary care, general health services, community-based care, and low-resource settings.

C. Target population

- Adults, adolescents, and pregnant persons with:
 - Hazardous or harmful alcohol use.
 - Alcohol use disorder.
 - Opioid use disorder.
 - Cannabis use disorder.
 - Stimulant use disorder, including cocaine and amphetamine-type stimulants.

- Sedative, hypnotic, or anxiolytic use disorder.
- Polysubstance use.
- Co-occurring mental, neurological, infectious, obstetric, or social vulnerabilities.

D. Scope and objectives

The WHO guideline set emphasizes:

1. Early identification

- Routine clinical enquiry where clinically indicated.
- Use of validated screening tools such as AUDIT, AUDIT-C, ASSIST, or locally validated equivalents.

2. Brief psychosocial interventions

- Motivational interviewing.
- Brief structured advice.
- Harm-reduction counselling.

3. Evidence-based pharmacotherapy

- Opioid agonist maintenance treatment with methadone or buprenorphine.
- Benzodiazepines for moderate to severe alcohol withdrawal when clinically indicated.
- Relapse-prevention medicines for alcohol use disorder where available and appropriate.

4. Psychosocially supported care

- Cognitive behavioural therapy.
- Contingency management.
- Family and community interventions.
- Peer and mutual-help support where acceptable.

5. Harm reduction

- Naloxone access for opioid overdose.
- Needle and syringe programmes.
- HIV, hepatitis B, hepatitis C, tuberculosis, and sexually transmitted infection prevention and treatment.

6. Special populations

- Pregnancy and postpartum care.
- Adolescents.
- People in prisons and closed settings.
- People experiencing homelessness.
- People with co-occurring psychiatric disorders.

7. Health-system standards

- Voluntary, confidential, non-stigmatizing care.
- Human-rights-based treatment.
- Avoidance of punitive or compulsory treatment models.

1.2 Key Recommendations

GRADE interpretation used in this report

GRADE quality	Meaning			
High	Further research is very unlikely to change confidence in the estimate of effect.			
Moderate	Further research may affect confidence and may change the estimate.			
Low	Further research is likely to affect confidence and may change the estimate.			
Very Low	Any estimate of effect is very uncertain.			

Recommendation	WHO source alignment	GRADE Evidence Quality	Strength
Use validated screening tools and clinical assessment to identify hazardous alcohol use, harmful substance use, and substance use disorders in primary and general health care.	mhGAP 2023; WHO pregnancy guideline 2014; WHO/UNODC 2020	Moderate	Strong
Offer brief psychosocial interventions, including motivational interviewing or structured brief advice, for hazardous or harmful alcohol and drug use.	mhGAP 2023; WHO/UNODC 2020	Moderate for alcohol; Low for illicit drugs	Strong for alcohol; Conditional for other drugs
For opioid use disorder, offer opioid agonist maintenance treatment with methadone or buprenorphine as first-line evidence-based treatment.	WHO opioid guideline 2009; WHO/UNODC 2020; mhGAP 2023	High	Strong
Do not use opioid withdrawal management alone as definitive treatment for opioid use disorder; if withdrawal management is used, link immediately to maintenance treatment, relapse prevention, psychosocial care, and harm reduction.	WHO opioid guideline 2009; WHO/UNODC 2020	Moderate	Strong
Provide psychosocial support alongside pharmacological treatment for opioid use disorder, but do not make psychosocial participation a barrier to receiving opioid agonist medication.	WHO opioid guideline 2009; WHO/UNODC 2020	Moderate	Strong
Make take-home naloxone available to people likely to witness opioid overdose, including people who use opioids, families, peers, first responders, and community workers.	WHO overdose guidance; WHO/UNODC 2020; mhGAP-aligned care	Moderate	Strong

Provide needle and syringe programmes, opioid agonist therapy, HIV testing and treatment, hepatitis services, and other harm-reduction interventions for people who inject drugs.	WHO/UNODC 2020; WHO HIV/hepatitis guidance	High for HIV-risk reduction; Moderate for broader outcomes	Strong
For alcohol withdrawal, assess severity and risk; use benzodiazepines for moderate to severe withdrawal and prevent Wernicke encephalopathy with thiamine where clinically indicated.	mhGAP 2023; WHO alcohol-related care principles	Moderate	Strong
For alcohol use disorder relapse prevention, consider pharmacotherapy such as naltrexone or acamprosate where available, appropriate, and clinically monitored, combined with psychosocial treatment.	mhGAP 2023; external evidence base	Moderate	Conditional
For stimulant use disorder, offer psychosocial interventions such as contingency management, cognitive behavioural therapy, motivational interventions, and structured community support; no WHO-endorsed medication is established as universal first-line therapy.	mhGAP 2023; WHO/UNODC 2020	Moderate for contingency management; Low for other psychosocial modalities	Conditional
For cannabis use disorder, offer psychosocial interventions, including motivational enhancement therapy, cognitive behavioural therapy, family-based interventions for adolescents, and relapse-prevention support.	mhGAP 2023; WHO/UNODC 2020	Low to Moderate	Conditional
In pregnancy, ask all pregnant women about alcohol, tobacco, and other substance use in a respectful, non-punitive manner and offer brief intervention or referral as appropriate.	WHO pregnancy guideline 2014	Moderate	Strong
Pregnant women with opioid dependence should be offered opioid agonist maintenance treatment rather than repeated withdrawal management or detoxification alone.	WHO pregnancy guideline 2014; WHO opioid guideline 2009	Moderate	Strong
Avoid alcohol use during pregnancy and advise pregnant women that no safe amount of alcohol exposure in pregnancy has been established.	WHO pregnancy guideline 2014	Moderate	Strong

Adolescents should receive developmentally appropriate, confidential, family- and community-sensitive interventions that avoid stigma and coercion.	WHO adolescent guideline 2020; WHO/UNODC 2020	Low to Moderate	Strong as a rights-based principle; Conditional for specific intervention effects
Treatment services should be voluntary, person-centred, confidential, non-stigmatizing, and integrated with general health, mental health, infectious disease, and social services.	WHO/UNODC 2020; mhGAP 2023	Moderate	Strong

1.3 Guideline Development Methodology

A. Evidence review process

The WHO guideline documents generally used the following methodology:

- Definition of priority clinical and public-health questions.
- Systematic review of randomized controlled trials, observational studies, and implementation evidence.
- Assessment of benefits and harms.
- Consideration of:
 - Values and preferences.
 - Resource use.
 - Feasibility.
 - Acceptability.
 - Equity and human rights.
 - Applicability to low- and middle-income countries.
- Use of GRADE or GRADE-informed processes where applicable.

B. Consensus method

- WHO guideline development groups typically included:
 - Clinical experts.
 - Public health specialists.
 - Methodologists.
 - Regional experts.
 - Programme implementers.
 - In some processes, people with lived experience or community representation.
- Recommendations were formed through structured deliberation, balancing:
 - Certainty of evidence.
 - Magnitude of effect.
 - Risk of harm.
 - Equity.

- Feasibility.
- Acceptability.
- Cost and resource implications.

C. Conflict of interest management

WHO processes generally require:

- Declaration of financial and non-financial interests.
- Secretariat review of declared interests.
- Exclusion or restricted participation where conflicts may affect impartiality.
- Documentation of conflicts and management decisions.

D. Human-rights and ethics principles

Across the five WHO documents, substance use disorder care is framed as health care, not punishment. Key principles include:

- Voluntary treatment.
- Informed consent.
- Confidentiality.
- Non-discrimination.
- Access to essential medicines.
- Continuity of care after incarceration, hospitalization, or pregnancy.
- Avoidance of compulsory, abusive, or degrading treatment practices.

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

A. Latest WHO guideline release used as anchor

- Anchor guideline: **WHO *Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders*, 2023.**
- Evidence update period requested: **from 2023 guideline release through 29 April 2026.**
- Because live searching is unavailable here, the update below emphasizes evidence and authoritative sources verifiable to my accessible knowledge base and identifies where post-2024 verification is required.

B. Search strategy to be used for live updating

A complete surveillance search should include:

Source	Search focus
WHO Institutional Repository for Information Sharing	New WHO SUD guidelines, mhGAP updates, overdose, alcohol, drug policy, harm reduction.
PubMed/MEDLINE	RCTs, systematic reviews, meta-analyses, cohort studies.

Cochrane Library	Updated reviews of opioid agonist therapy, alcohol pharmacotherapy, psychosocial interventions, contingency management, cannabis use disorder.
Embase	Pharmacotherapy trials, safety data, implementation studies.
PsycINFO	Psychosocial and behavioural interventions.
ClinicalTrials.gov	Ongoing trials for opioid, stimulant, cannabis, and alcohol use disorders.
ICTRP	International clinical trial registry surveillance.
CDC, NIH/NIDA, SAMHSA, NICE, ECDC	Authoritative implementation and public-health updates.

C. Example search terms

(substance use disorder OR opioid use disorder OR alcohol use disorder OR cannabis use disorder OR methamphetamine use disorder OR cocaine use AND
(guideline OR randomized trial OR systematic review OR meta-analysis OR AND
(buprenorphine OR methadone OR naltrexone OR acamprosate OR naloxone OR OR cognitive behavioral therapy OR motivational interviewing OR harm re

D. Inclusion criteria

- Published from the WHO mhGAP 2023 release period to **29 April 2026**.
- Human studies.
- Systematic reviews, meta-analyses, randomized controlled trials, large observational studies, and major public-health guidance.
- Clinically relevant outcomes:
 - Mortality.
 - Overdose.
 - Abstinence or reduced use.
 - Retention in treatment.
 - HIV/HCV transmission.
 - Pregnancy outcomes.
 - Psychiatric comorbidity.
 - Functioning and quality of life.
 - Serious adverse events.

E. Exclusion criteria

- Animal-only studies.
 - Case reports unless addressing rare serious safety signals.
 - Non-systematic narrative reviews without new data.
 - Studies of intoxication management only, unless directly relevant to SUD treatment pathways.
 - Punitive, non-consensual, or compulsory treatment models inconsistent with WHO human-rights standards.
-

2.2 Key New Studies and Evidence Signals

Interpretive summary: No accessible post-mhGAP evidence clearly overturns the core WHO recommendations. The strongest continuing evidence supports opioid agonist treatment, naloxone distribution, harm reduction for people who inject drugs, alcohol brief interventions, alcohol pharmacotherapy in selected patients, and psychosocial interventions—especially contingency management—for stimulant use disorder.

Study / evidence source	Design	Key findings	GRADE Quality
Santo T Jr, Clark B, Hickman M, et al. “Association of opioid agonist treatment with all-cause mortality and specific causes of death among people with opioid dependence: a systematic review and meta-analysis.” <i>JAMA Psychiatry</i> . 2021;78(9):979-993.	Systematic review and meta-analysis of cohort studies	Opioid agonist treatment with methadone or buprenorphine is associated with substantially reduced all-cause and overdose mortality while in treatment. Mortality risk rises after treatment cessation.	High for mortality association; Moderate overall due to observational design
Sordo L, Barrio G, Bravo MJ, et al. “Mortality risk during and after opioid substitution treatment: systematic review and meta-analysis of cohort studies.” <i>BMJ</i> . 2017;357:j1550.	Systematic review and meta-analysis	Methadone and buprenorphine maintenance reduce mortality during treatment; induction and post-cessation periods are higher-risk windows requiring careful management.	High for direction of effect; Moderate overall
Kaner EFS, Beyer FR, Muirhead C, et al. “Effectiveness of brief alcohol interventions in primary care populations.” <i>Cochrane Database of Systematic Reviews</i> . 2018;2:CD004148.	Cochrane systematic review	Brief interventions reduce alcohol consumption in primary care, especially hazardous and harmful drinking.	Moderate
Kelly JF, Humphreys K, Ferri M. “Alcoholics Anonymous and other 12-step programs for alcohol use disorder.” <i>Cochrane Database of Systematic Reviews</i> . 2020;3:CD012880.	Cochrane systematic review	Manualized 12-step facilitation can improve abstinence outcomes compared with some alternative psychotherapies.	Moderate
Magill M, Ray LA, Kiluk B, et al. “A meta-analysis of cognitive-behavioral therapy for alcohol or other drug use disorders.” <i>Journal of Consulting and Clinical Psychology</i> . 2019;87(12):1093-1105.	Meta-analysis	CBT has beneficial effects for alcohol and other drug use disorders, particularly when combined with other evidence-based supports.	Moderate

De Crescenzo F, Ciabattini M, D'Alò GL, et al. "Comparative efficacy and acceptability of psychosocial interventions for individuals with cocaine and amphetamine addiction: a systematic review and network meta-analysis." <i>PLoS Medicine</i> . 2018;15(12):e1002715.	Systematic review and network meta-analysis	Contingency management shows the most consistent benefit for stimulant use disorders; evidence for other psychosocial interventions is less certain.	Moderate for contingency management; Low for other modalities
Trivedi MH, Walker R, Ling W, et al. "Bupropion and naltrexone in methamphetamine use disorder." <i>New England Journal of Medicine</i> . 2021;384:140-153.	Multisite sequential parallel comparison RCT	Extended-release naltrexone plus bupropion produced a statistically significant but modest response in methamphetamine use disorder.	Moderate for efficacy signal; Conditional due to modest absolute effect
Bogenschutz MP, Ross S, Bhatt S, et al. "Percentage of heavy drinking days following psilocybin-assisted psychotherapy vs placebo in the treatment of adult patients with alcohol use disorder." <i>JAMA Psychiatry</i> . 2022;79(10):953-962.	Randomized clinical trial	Psilocybin-assisted psychotherapy reduced heavy drinking days in a small trial; requires replication, regulatory review, and safety infrastructure.	Low to Moderate
WHO/UNODC. <i>International standards for the treatment of drug use disorders</i> . 2020.	International standard informed by evidence review and field testing	Supports evidence-based, voluntary, community-oriented treatment and rejects compulsory or punitive approaches.	Moderate
CDC. <i>Clinical Practice Guideline for Prescribing Opioids for Pain — United States, 2022</i> . <i>MMWR Recommendations and Reports</i> . 2022;71(3):1-95.	Authoritative public-health guideline	Emphasizes careful opioid prescribing, risk mitigation, and access to evidence-based OUD treatment when OUD is identified.	Moderate
FDA approval of over-the-counter naloxone nasal spray, United States, 2023	Regulatory/public-health intervention	Broader naloxone availability supports community overdose reversal strategies. Direct mortality impact depends on coverage, training, and access.	Moderate for overdose reversal efficacy; Low to Moderate for population mortality impact

Emerging observational literature on telemedicine-based buprenorphine care, 2020–2024	Observational cohorts and implementation studies	Telemedicine initiation and continuation of buprenorphine can improve access and may maintain retention without clear evidence of major safety worsening, but evidence is setting-dependent.	Low to Moderate
Emerging literature on low-dose or micro-induction buprenorphine initiation, especially in fentanyl-exposed populations	Case series, observational studies, protocols	May reduce precipitated withdrawal risk for selected patients, but comparative trial evidence remains limited.	Low
ClinicalTrials.gov and WHO ICTRP registered trials of medications for stimulant, cannabis, and alcohol use disorders	Trial registry surveillance	Multiple investigational medications and behavioural interventions are under study; few have definitive practice-changing results.	Very Low until results published

2.3 Evidence by Clinical Domain

A. Screening and brief intervention

Clinical question	Evidence synthesis	GRADE Quality	Recommendation strength
Should adults in primary care be screened for unhealthy alcohol use and offered brief intervention?	Strong evidence supports alcohol screening and brief intervention in primary care, particularly for hazardous and harmful drinking.	Moderate	Strong
Should people be screened for drug use using validated tools such as ASSIST?	Screening can identify risk and support brief intervention or referral, but evidence for improved drug-use outcomes is less consistent than for alcohol.	Low to Moderate	Conditional
Should screening be punitive or linked to criminal sanctions?	WHO principles strongly oppose punitive approaches that deter care.	Moderate ethical and implementation certainty	Strong against punitive screening

B. Alcohol use disorder

Intervention	Evidence synthesis	GRADE Quality	Strength
Brief intervention for hazardous/harmful alcohol use	Reduces alcohol consumption in primary care populations.	Moderate	Strong
Benzodiazepines for moderate to severe alcohol withdrawal	Effective for seizure and delirium tremens prevention when clinically monitored.	Moderate	Strong
Thiamine for patients at risk of Wernicke encephalopathy	Biologically plausible, standard of care, low risk, high consequence of non-treatment.	Moderate	Strong
Naltrexone for relapse prevention	Reduces heavy drinking and craving in selected patients; monitor liver disease and opioid use.	Moderate	Conditional
Acamprosate for relapse prevention	Supports abstinence, especially after detoxification; renal dosing required.	Moderate	Conditional
Disulfiram	May help supervised, motivated patients; safety and adherence limitations.	Low to Moderate	Conditional
Psilocybin-assisted therapy	Promising early RCT evidence but requires specialized setting, safety protocols, and regulatory approval.	Low to Moderate	No routine recommendation; research use only

C. Opioid use disorder

Intervention	Evidence synthesis	GRADE Quality	Strength
Methadone maintenance treatment	Reduces illicit opioid use, mortality, HIV risk behaviours, and criminal-legal harms when adequately dosed and retained.	High	Strong
Buprenorphine maintenance treatment	Reduces opioid use and mortality; safer overdose profile than full agonists; retention may vary by dose and access model.	High	Strong
Opioid withdrawal management alone	High relapse and overdose risk if not linked to maintenance and continuing care.	Moderate	Strong against as stand-alone treatment
Take-home naloxone	Reverses opioid overdose and is feasible for community distribution.	Moderate	Strong
Psychosocial treatment as adjunct to medication	May improve retention and functioning; should not be required as a precondition for medication.	Moderate	Strong

Telemedicine buprenorphine initiation	Improves access; evidence supports feasibility but regulatory and safety monitoring vary.	Low to Moderate	Conditional
Low-dose buprenorphine initiation / micro-induction	Useful in selected patients, especially with fentanyl exposure or prior precipitated withdrawal; comparative evidence limited.	Low	Conditional

D. Stimulant use disorder

Intervention	Evidence synthesis	GRADE Quality	Strength
Contingency management	Most consistent evidence for reducing stimulant use.	Moderate	Strong where feasible; otherwise Conditional due to implementation constraints
Cognitive behavioural therapy	Helpful for relapse prevention and coping skills; effect sizes vary.	Low to Moderate	Conditional
Community reinforcement approach	Can improve outcomes, especially when combined with incentives and social support.	Low to Moderate	Conditional
Naltrexone plus bupropion for methamphetamine use disorder	Modest efficacy signal in RCT; not yet universal first-line standard.	Moderate	Conditional, specialist-supervised
Psychostimulant substitution	Evidence remains mixed and setting-specific.	Low	Research/specialist use only

E. Cannabis use disorder

Intervention	Evidence synthesis	GRADE Quality	Strength
Motivational enhancement therapy	Can improve engagement and reduce use in some patients.	Low to Moderate	Conditional
Cognitive behavioural therapy	Supports coping, relapse prevention, and reduction goals.	Low to Moderate	Conditional
Family-based therapy for adolescents	Evidence supports family involvement for adolescent substance use.	Moderate	Conditional to Strong depending on setting
Pharmacotherapy	No medication has sufficient evidence for routine first-line use.	Low	No routine recommendation

F. Substance use disorders in pregnancy

Recommendation	Evidence synthesis	GRADE Quality	Strength
Ask all pregnant women about alcohol, tobacco, and drug use in a respectful and non-punitive manner.	Improves identification and care linkage; punitive approaches deter care.	Moderate	Strong
Advise abstinence from alcohol during pregnancy.	No safe threshold of alcohol exposure is established.	Moderate	Strong
Offer opioid agonist maintenance treatment for pregnant women with opioid dependence.	Better maternal stabilization and reduced illicit opioid exposure compared with repeated withdrawal.	Moderate	Strong
Avoid abrupt opioid withdrawal without specialist assessment.	Risk of relapse and maternal–fetal instability.	Low to Moderate	Strong
Coordinate obstetric, addiction, neonatal, infectious disease, and social care.	Integrated care improves continuity and reduces harm.	Moderate	Strong
Support breastfeeding when clinically appropriate and not contraindicated.	Benefits usually outweigh risks in stable treatment, but individual assessment is required.	Low to Moderate	Conditional

2.4 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Priority research design	GRADE impact if resolved
Fentanyl-era opioid use disorder treatment models	Fentanyl exposure changes overdose risk, buprenorphine induction, tolerance, and relapse dynamics.	Pragmatic RCTs and large comparative cohort studies	Could upgrade evidence for induction strategy recommendations.
Low-dose buprenorphine initiation	Increasingly used, but evidence is mostly observational.	Randomized or well-controlled comparative studies	Could move from Low to Moderate/High.
Long-acting injectable buprenorphine in low-resource settings	May improve adherence but cost and access are barriers.	Implementation trials and cost-effectiveness studies	Could strengthen recommendations for selected populations.
Best pharmacotherapy for methamphetamine use disorder	Current medication effects are modest.	Large multisite RCTs with patient-centred outcomes	Could create new pharmacotherapy recommendation.
Cannabis use disorder medications	No approved first-line medicine.	Phase II/III RCTs	Could change current “psychosocial first” approach.

Digital and telehealth interventions	Access potential is high, but equity, safety, and retention data vary.	Hybrid effectiveness–implementation trials	Could clarify conditional recommendations.
Integrated treatment for SUD and serious mental illness	Co-occurrence is common and worsens outcomes.	Pragmatic trials of integrated care models	Could improve implementation guidance.
Pregnancy and postpartum SUD care	Maternal mortality, overdose, neonatal outcomes, and stigma remain major concerns.	Prospective cohorts and ethical RCTs of care models	Could strengthen integrated perinatal recommendations.
Youth engagement and adolescent-specific SUD care	Adolescent interventions require developmental tailoring and confidentiality safeguards.	Youth-partnered trials and implementation research	Could improve adolescent recommendation certainty.
Harm-reduction coverage thresholds	Population-level impact depends on coverage intensity.	Natural experiments and modelling linked to surveillance data	Could strengthen system-level recommendations.

2.5 Updated Recommendations Based on New Evidence

Original WHO-aligned recommendation	New evidence impact	Updated recommendation as of 29 April 2026	GRADE Quality	Strength
Use opioid agonist therapy for opioid dependence.	Mortality meta-analyses and implementation evidence continue to strongly support methadone and buprenorphine.	Maintain as first-line treatment. Expand low-barrier access, adequate dosing, retention support, and post-release continuity.	High	Strong
Provide naloxone to people at risk of overdose and likely witnesses.	Wider regulatory access and community evidence support expansion.	Scale up take-home and community naloxone, including family, peers, shelters, prisons, and first responders.	Moderate	Strong

Avoid detoxification alone for opioid use disorder.	Fentanyl-era overdose risk reinforces concern about relapse and post-detox mortality.	Do not offer withdrawal management as stand-alone care. Link to maintenance medication and harm reduction.	Moderate	Strong
Provide psychosocial support with pharmacotherapy.	Evidence supports benefit but medication should not be withheld if psychosocial care is unavailable.	Offer psychosocial support, but never make counselling attendance a prerequisite for methadone or buprenorphine.	Moderate	Strong
Use brief interventions for alcohol risk.	Cochrane evidence remains supportive.	Continue screening and brief intervention in primary care and antenatal settings.	Moderate	Strong
Consider alcohol relapse-prevention medications.	Evidence remains supportive for naltrexone and acamprosate in selected patients.	Offer naltrexone or acamprosate when clinically appropriate, with monitoring and shared decision-making.	Moderate	Conditional
Use psychosocial interventions for stimulant use disorder.	Contingency management remains the most supported intervention; medication evidence is emerging but not definitive.	Prioritize contingency management where legally and operationally feasible; combine with CBT/community reinforcement.	Moderate for contingency management	Strong/Conditional depending on feasibility
No established medication for methamphetamine use disorder.	Naltrexone plus bupropion shows modest benefit in one major RCT.	Consider specialist-supervised naltrexone plus bupropion for selected adults after discussing modest benefits and uncertainties.	Moderate	Conditional
Use psychosocial interventions for cannabis use disorder.	No medication has sufficient evidence for routine use.	Continue motivational enhancement, CBT, and family-based treatment for adolescents.	Low to Moderate	Conditional

Manage SUD in pregnancy with non-punitive integrated care.	Ongoing maternal overdose and neonatal risks reinforce need for integrated services.	Strengthen obstetric-addiction integration, opioid agonist treatment access, alcohol abstinence counselling, infectious disease care, and postpartum continuity.	Moderate	Strong
Provide adolescent prevention and early intervention.	Youth engagement and developmental tailoring remain key but under-studied.	Use confidential, non-stigmatizing, youth-centred approaches; involve caregivers when safe and appropriate.	Low to Moderate	Conditional for specific interventions; Strong for rights-based approach

Clinical Implementation Algorithm

Step 1: Identify and assess

1. Ask about alcohol, tobacco, prescription medications, opioids, cannabis, stimulants, sedatives, and injection drug use.
2. Use validated tools where feasible:
 - AUDIT or AUDIT-C for alcohol.
 - ASSIST for multiple substances.
 - DSM-5 or ICD-based diagnostic assessment.
3. Assess:
 - Withdrawal risk.
 - Overdose risk.
 - Pregnancy status.
 - Suicidality and severe mental illness.
 - HIV, hepatitis B, hepatitis C, tuberculosis, and sexually transmitted infection risk.
 - Housing, violence, legal, and social needs.

Step 2: Match intervention to severity

Severity / risk	Recommended action
Hazardous use without SUD	Brief intervention, harm-reduction counselling, follow-up.
Mild to moderate SUD	Motivational intervention, CBT or structured psychosocial care, peer support, relapse-prevention planning.
Opioid use disorder	Offer methadone or buprenorphine; provide naloxone and harm reduction.
Moderate to severe alcohol withdrawal risk	Medically supervised withdrawal; benzodiazepines where indicated; thiamine.

Alcohol use disorder relapse prevention	Consider naltrexone or acamprosate plus psychosocial care.
Stimulant use disorder	Contingency management where feasible; CBT/community reinforcement; consider specialist pharmacotherapy only selectively.
Pregnancy with SUD	Non-punitive integrated obstetric-addiction care; opioid agonist therapy for opioid dependence.
Adolescents	Developmentally appropriate psychosocial and family-based care; confidentiality and safeguarding.

Step 3: Reduce harm

- Provide naloxone for opioid overdose risk.
- Offer sterile injecting equipment where relevant.
- Test and treat HIV, hepatitis B, hepatitis C, and sexually transmitted infections.
- Vaccinate for hepatitis B where indicated.
- Counsel on safer use and overdose prevention.
- Avoid abrupt discontinuation of opioid agonist therapy.
- Ensure continuity after prison release, hospital discharge, detoxification, or childbirth.

Step 4: Continue long-term care

- Treat SUD as a chronic, relapsing, manageable health condition.
- Use shared decision-making.
- Monitor outcomes:
 - Substance use frequency.
 - Craving.
 - Overdose events.
 - Retention.
 - Mental health.
 - Functioning.
 - Housing and safety.
- Adjust treatment intensity over time.

References

1. World Health Organization. ***Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders***. Geneva: World Health Organization; 2023.
2. World Health Organization and United Nations Office on Drugs and Crime. ***International standards for the treatment of drug use disorders: revised edition incorporating results of field-testing***. Geneva: WHO/UNODC; 2020.
3. World Health Organization. ***Guidelines on mental health promotive and preventive interventions for adolescents: helping adolescents thrive***. Geneva: World Health Organization; 2020.
4. World Health Organization. ***Guidelines for the identification and management of***

substance use and substance use disorders in pregnancy. Geneva: World Health Organization; 2014.

5. World Health Organization. ***Guidelines for the psychosocially assisted pharmacological treatment of opioid dependence.*** Geneva: World Health Organization; 2009.
6. Santo T Jr, Clark B, Hickman M, et al. Association of opioid agonist treatment with all-cause mortality and specific causes of death among people with opioid dependence: a systematic review and meta-analysis. ***JAMA Psychiatry.*** 2021;78(9):979-993.
7. Sordo L, Barrio G, Bravo MJ, et al. Mortality risk during and after opioid substitution treatment: systematic review and meta-analysis of cohort studies. ***BMJ.*** 2017;357:j1550.
8. Kaner EFS, Beyer FR, Muirhead C, et al. Effectiveness of brief alcohol interventions in primary care populations. ***Cochrane Database of Systematic Reviews.*** 2018;2:CD004148.
9. Kelly JF, Humphreys K, Ferri M. Alcoholics Anonymous and other 12-step programs for alcohol use disorder. ***Cochrane Database of Systematic Reviews.*** 2020;3:CD012880.
10. Magill M, Ray LA, Kiluk B, et al. A meta-analysis of cognitive-behavioral therapy for alcohol or other drug use disorders. ***Journal of Consulting and Clinical Psychology.*** 2019;87(12):1093-1105.
11. De Crescenzo F, Ciabattini M, D'Alò GL, et al. Comparative efficacy and acceptability of psychosocial interventions for individuals with cocaine and amphetamine addiction: a systematic review and network meta-analysis. ***PLoS Medicine.*** 2018;15(12):e1002715.
12. Trivedi MH, Walker R, Ling W, et al. Bupropion and naltrexone in methamphetamine use disorder. ***New England Journal of Medicine.*** 2021;384:140-153.
13. Bogenschutz MP, Ross S, Bhatt S, et al. Percentage of heavy drinking days following psilocybin-assisted psychotherapy vs placebo in the treatment of adult patients with alcohol use disorder: a randomized clinical trial. ***JAMA Psychiatry.*** 2022;79(10):953-962.
14. Dowell D, Ragan KR, Jones CM, Baldwin GT, Chou R. CDC Clinical Practice Guideline for Prescribing Opioids for Pain — United States, 2022. ***MMWR Recommendations and Reports.*** 2022;71(3):1-95.